Q1. Discuss aetiology, clinical features and management of Carcinoma of Tongue. Or

Describe signs, symptoms and management of carcinoma of posterior third of tongue. Or

Predisposing factors – carcinoma of a tongue.

Q2. Differential Diagnosis of Ulcers Over the Tongue

Q3. Describe aetiology, clinical features diagnosis and management of Carcinoma of the Cheek. How will you manage if it has involved the mandible .

Q4.Lingual Thyroid

Q5. Deep Vein Thrombosis (DVT)

Q6. Enlist causes of secondary in the neck with occult primary. How will you manage such a case.

Q7. Differential Diagnosis of Abscess and Cellulitis

Q8. Differential Diagnosis of Non-Healing Ulcer

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Describe signs, symptoms and management of carcinoma of posterior third of tongue. Or

Predisposing factors - carcinoma of a tongue.

Carcinoma of the Tongue

Premalignant Conditions

- **Leukoplakia**

- **Erythroplakia**

Six Ss

- **Smoking**
- **Syphilis**
- **Spices**
- **Spirits**
- **Sharp tooth**
- **Sepsis (betel nut/tobacco)**

Types of Carcinoma

- **Pathological Types:**

1. **Non-healing ulcer** (lateral border)
2. **Proliferative growth**
3. **Frozen tongue**
4. **Fissure variety**

- **Histological Types:**

1. **Squamous cell carcinoma** (most common)
2. **Adenocarcinoma** (minor salivary glands)
3. **Melanomas**

- **Site of Involvement:**

1. **Lateral margin** (47-50%)
2. **Posterior third** (20%)
3. **Dorsum** (6.5%)
4. **Ventral surface** (9%)

Clinical Features

1. Affects **elderly**
2. **Pain** (infection, ulceration, lingual nerve)
3. **Excessive salivation**
4. **Dysphagia**
5. **Ankyloglossia**

6. **Foetororis** (halitosis)
7. **Palpable lymph nodes** (hard, nodular)

Spread

- **Local:**

1. Anterior 2/3 doesn't cross midline
2. Posterior third spreads to tonsils, epiglottis

- **Lymphatic:**

1. **Tip → submental**
2. **Lateral margin → submandibular, juguloomohyoid**
3. **Posterior third → pharyngeal, cervical nodes**

1. **Blood**: Rare, late

Investigations

- **Edge biopsy** (posterior third)
- **FNAC** of lymph nodes
- **Indirect laryngoscopy**
- **CT scan** (posterior third)
- **Chest X-ray** (bronchopneumonia)

Treatment

1. **Surgery** (wide excision, glossectomy, Commando's operation)
2. **Radiotherapy** (teletherapy, interstitial)
3. **Chemotherapy** (Methotrexate)

Secondary Lymph Nodes

- **Prophylactic block dissection**
- **En bloc radical neck dissection**
- **Palliative radiotherapy/chemotherapy**

Prognosis

1. **5-year survival rate: ~25%**
2. **Cause of death**: Cachexia, aspiration, hemorrhage, asphyxia

Q2. Differential Diagnosis of Ulcers Over the Tongue
Various ulcers that can occur on the tongue include:

1. **Aphthous ulcer**
2. **Dental ulcer** (traumatic)
3. **Chronic non-specific ulcer**
4. **Syphilitic ulcer**
5. **ulcer**
6. **Carcinomatous ulcer**
7. **Herpetic ulcer**
8. **Glossitis causing ulcers**
9. **Post-pertussis ulcer** (whooping cough)

Painful vs. Painless Ulcers

- **Painful ulcers**:
    1. **Aphthous ulcers**
    2. **Tubercular ulcers**
    3. **Dental ulcers**
- **Painless ulcers**:
    1. **Carcinomatous ulcers**
    2. **Gummatous ulcers**
    3. **Systemic diseases**

i. Aphthous Ulcers

The term **aphthous ulcer** refers to three specific conditions:

3. **Recurrent Aphthous Minor**
4. **Recurrent Aphthous Major**
5. **Herpetiform Aphthous Ulceration**

a. Recurrent Aphthous Minor

- Appears in **crops** of multiple, painful ulcers.
- More common in **females**, especially during menstruation.
- **Size**: 0.5 cm, round or oval in shape.
- **Appearance**: Yellow floor with red **erythematous** margin.
- **Healing time**: 10-14 days.
- **Treatment**:
    - **Chlorhexidine gluconate** mouthwash.

- **Choline salicylate gel** for temporary relief.
    - **Vitamin B complex** (placebo effect).

b. Recurrent Aphthous Major

- Larger, deeper, and more painful than minor ulcers.
- Due to **viral infection**.
- Takes longer to heal, and heals with **scarring**.

c. Herpetiform Aphthous Ulcers

- Small and occur in **crops** of many ulcers.
- Not caused by **herpes simplex virus**.
- Treatment:
    - **Chlorhexidine** or **tetracycline mouthwashes** lead to rapid healing.

ii. Dental Ulcer (Traumatic)

- Caused by **broken tooth**, **sharp tooth**, or **ill-fitting dentures**.
- **Very painful** ulcers.
- Commonly occur on the **lateral margin** of the tongue.
- Healing occurs once the **tooth is extracted**.

iii. Chronic Non-Specific Ulcer

- Solitary, **painless**, and slightly indurated ulcer.
- Commonly found on the **anterior part of the tongue**.
- **Symptoms**: Burning sensation while eating spicy food.
- Treatment: **Mouthwashes** and **cauterization** may be sufficient.

iv. Syphilitic or Gummatous Ulcer

- **Snail track ulcers** seen in the **2nd stage** of syphilis.
- **Gumma** occurs in the **tertiary stage** of syphilis.
- **Primary chancre** can occur on the **tip** of the tongue, usually in the **midline**.
- **Mucous patches** may occur anywhere on the tongue.
- **Gumma**: Firm swelling in the midline of the anterior two-thirds of the tongue.
- **Ulcer characteristics**: **Non-tender** with punched-out edges and **wash leather slough**.
- These ulcers are **rare** today due to the use of **penicillin**.

v. Tubercular Ulcer of Tongue

- The tongue is **not a common site** for tuberculosis, but it affects the **tip** of the tongue.
- Seen in **young adults** with **pulmonary tuberculosis**.
- **Multiple shallow ulcers** with **undermined edges** are common.
- **Painful ulcers** with **enlarged regional lymph nodes**.
- There may be **matted lymph nodes** in the neck.
- Occurs in **fulminating pulmonary tuberculosis**.
- **Ulcers** are sometimes **multiple** with **serous discharge**.
- **Treatment**: Follow the **antituberculous** treatment regimen.
    - **Antitubercular treatment** will heal these ulcers.

vi. Systemic Diseases

Ulcers may appear on the tongue in the following **systemic diseases**:

- **Pemphigus**
- **Systemic lupus erythematosus (SLE)**
- **Lichen planus**

vii. Herpetic Ulcer

- Caused by **herpetic infection** of the **lingual nerve**.
- **Acute neuralgic pain** usually precedes the appearance of vesicles.
- The vesicles may **rupture** into **multiple small painful ulcers**.
- **Herpes zoster** can be treated with **antiviral drugs**.

viii. Glossitis Causing Ulcers

- **Chronic superficial glossitis** is seen in **smokers** (known as **smoker's ulcer**).
- **Ulcers** are **superficial**, **multiple**, and **hyperemic**.
- These ulcers are **painful**.

ix. Post-Pertussis Ulcer (Whooping Cough)

- Occurs in **children** due to **repeated coughing**.
- Typical location of the ulcer is on the **undersurface of the tongue** and on the **frenulum**, which helps clinch the diagnosis.

Q3. Describe aetiology, clinical features diagnosis and management of Carcinoma of the Cheek. How will you manage if it has involved the mandible .

- **Most common type**: **Squamous cell carcinoma**.
- **Occasionally**: Adenocarcinoma from minor salivary glands or rarely melanoma.
- **Common in India**: Due to **tobacco quid** kept in the cheek pouch.

Aetiology

- All **S**: **Smoking**, **spirit**, **syphilis**, **sharp tooth**, **spices**.
- **Premalignant conditions**:
    - **Leukoplakia**
    - **Erythroplakia**
    - **Hyperplastic candidiasis**
    - **Submucosal fibrosis**
    - **Betel nut chewing (pan)**.

Pathological Types

1. **Non-healing ulcer**.
2. **Exophytic growth** (Verrucous carcinoma).
3. **Infiltrative lesion**: Slowly involves adjacent structures.

Clinical Features

- **Non-healing ulcer** or **cauliflower-like growth**.
- **Everted edges** and **induration** are typical features.
- **Bleeding** on touch.
- **Pain** occurs when involving skin, bone, or secondary infection.
- **Fixity** to underlying structures (e.g., mandible).

- **Retromolar trigone involvement**: Advanced disease.
- **Trismus** and **dysphagia** indicate pterygoid involvement.
- **Halitosis** is characteristic.
- **Lymph node involvement**: Submandibular and upper deep cervical. Initially mobile, later fixed.

Investigations

1. **Wedge biopsy** from the edge of the lesion (keratin pearls are a feature).
2. **OPG** of the mandible to check for extension.
3. **FNAC** from lymph nodes.
4. **CT scans** to assess tumor extension and secondaries.

Treatment

Treatment may be **curative** or **palliative**.

- **Surgery**:
    1. **Small superficial ulcer** (T1, T2): Wide excision followed by **split skin graft** (SSG).
    2. **Early growth without bone involvement**: **Curative radiotherapy** (e.g., Caesium137 or Iridium 192) or wide excision with 3 cm clearance.
    3. **Infiltrative ulcer**: Wide excision followed by **flap reconstruction** (e.g., **PMMC flap**).
- **Radiotherapy**:
    1. **T1 and T2 lesions**.
    2. **Patients unfit for surgery**.
    3. **Lesions near the commissure**.

    Types of radiotherapy:
    - **External radiotherapy**: High total dose (6000-8000 cGy).
    - **Interstitial radiotherapy**: For infiltrative lesions (using Caesium 137 or Iridium wires).
- **Chemotherapy**:
  Drugs: **Methotrexate**, **Vincristine**, **Bleomycin**, **Adriamycin**.
  Often given **intra-arterially** via the **external carotid artery** using an **arterial pump** or by increasing the drip height to increase pressure.

Advanced Carcinoma of the Cheek

- **T3 and T4 lesions**: Surgery followed by **postoperative radiotherapy**.
- **Full thickness resection**: Large defects can be repaired using:
    - **Split skin graft**.

- **Deltopectoral flap** (based on the 2nd perforator of the internal mammary artery).
- **Forehead flap** (based on the superficial temporal artery).
- **Pectoralis major myocutaneous flap**.
- **Mandible reconstruction** using **cortical bone graft**, **rib**, **fibula**, or **synthetic material**.
- **Free flaps** (microvascular).

Q4.Lingual Thyroid

- **Description**:
  - A **round, reddish swelling** seen at the **back of the tongue**, specifically at the **foramen caecum**.
- **Development**:
  - The thyroid develops from the **thyroglossal bud**, which originates from the **foramen caecum** and descends into the neck.
  - This descent can be **arrested** at any stage, and if it stops at the **foramen caecum**, it forms a **lingual thyroid**.
- **Important Note**:
  - The **lingual thyroid** is the **only existing thyroid tissue** in some individuals.
  - If it needs to be removed, it should be **reimplanted** elsewhere to avoid thyroid hormone deficiency.
- **Potential Complications**:
  - **Haemorrhage** (bleeding).
  - **Respiratory obstruction** (difficulty in breathing).
  - **Dysphagia** (difficulty in swallowing).
  - **Speech impairment** (difficulty in speaking).
- **Diagnosis**:
  1. **Radioisotope study**:
     - Shows **iodine uptake** by the lingual thyroid and assesses the **status of the thyroid** in its normal location (in the neck).
  2. **Ultrasound of the neck**:
     - Helps to confirm the **absence of normal thyroid tissue** in the neck and locate the lingual thyroid.
- **Treatment**:
  1. **L-thyroxine** (oral medication):

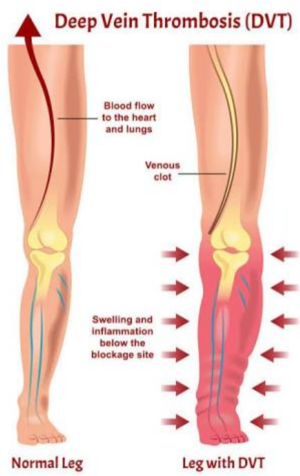
- Can help **reduce the swelling** of the lingual thyroid.

2. **Surgical excision**:
    - If necessary, the lingual thyroid can be surgically removed.
3. **Radioisotope therapy**:
    - Used for **ablation** (destruction) of the thyroid tissue if surgery is not preferred.

Key Points:

- **Lingual thyroid** is the only thyroid tissue in some cases.
- Careful consideration is needed if removal is necessary, with reimplantation advised.
- It can cause significant symptoms like **difficulty in breathing** and **swallowing** if not treated.

Q5. Deep Vein Thrombosis (DVT)



Definition:

- **DVT** refers to the **formation of a thrombus (clot)** in the **deep venous system**, commonly affecting the **lower extremities** and **pelvis**.

Causes (Etiology)

- **Virchow's Triad**:
    1. **Venous stasis**:
        - Prolonged immobility (bed rest, long flights).
    2. **Hypercoagulability**:
        - Cancer, pregnancy, oral contraceptives.
    3. **Endothelial injury**:

    - Trauma, surgery.

Risk Factors

- Recent surgery or trauma.
- Prolonged immobility (e.g., travel, hospitalization).
- Cancer, pregnancy, obesity.
- Use of oral contraceptives or hormone replacement therapy.
- Smoking.
- Previous history of DVT or family history.

Pathophysiology

1. **Clot formation** begins in the deep veins due to **stasis, hypercoagulability**, or **injury**.
2. Clots can obstruct blood flow, leading to **swelling, pain**, and **inflammation**.
3. There is a risk of **pulmonary embolism (PE)** if a clot dislodges and travels to the lungs.

Clinical Features

- **Pain**: Typically in the calf or thigh, worsens with movement.
- **Swelling**: Unilateral leg swelling.
- **Redness and warmth**: Over the affected area.
- **Homan's Sign**: Pain in the calf on dorsiflexion of the foot (not very specific).

Complications

1. **Pulmonary Embolism (PE)**: Life-threatening complication where the clot travels to the lungs.
2. **Post-thrombotic syndrome**: Chronic leg pain and swelling.
3. **Recurrent DVT**: Increased risk after the first event.

Investigations

1. **D-dimer test**: Elevated levels indicate clot formation.
2. **Doppler Ultrasound**: Main diagnostic test to visualize the thrombus.
3. **CT/MRI venography**: Used in complex cases.

Treatment of DVT

1. General Measures:

- **Bed rest**: Legs elevated to **15°** to reduce swelling.
- **Graduated elastic stockings**: Prevent further clot formation and improve venous return.
- **Physiotherapy**: Once the acute phase resolves to prevent long-term complications.

2. Medications:

- **Heparin (unfractionated or low molecular weight)**:
  - Immediate anticoagulation to prevent clot progression.
- **Warfarin**:
  - Oral anticoagulant given for **at least 3 months**.
  - Requires INR monitoring (goal INR: 2-3).
- **Low Molecular Weight Heparin (LMWH)**:
  - Preferred for initial treatment due to better safety and ease of use.
- **Thrombolytic agents (Streptokinase)**:
  - Used in selected cases during the early stages of DVT.

3. Surgical Management:

- **Thrombectomy**:
  - Rarely performed; indicated in massive clots or if anticoagulation is contraindicated.
- **IVC (Inferior Vena Cava) Filter**:
  - Used in patients at high risk of pulmonary embolism when anticoagulation is contraindicated.

Prevention

- Early mobilization after surgery or prolonged bed rest.
- Use of compression stockings during travel or immobility.
- Prophylactic anticoagulation in high-risk patients.
- Adequate hydration and avoiding smoking.

Key Points

- **Prompt diagnosis and treatment** of DVT can prevent life-threatening complications like **pulmonary embolism**.
- **Regular follow-up** is essential to monitor anticoagulation therapy and prevent recurrence.

Q6. Enlist causes of secondary in the neck with occult primary. How will you manage such a case.

Definition:

- **Secondaries in the neck** with **occult primary** refers to **metastatic neck lymph nodes** with the **primary tumor location unknown**.

Causes

1. **Head and Neck Cancers**:
   - **Nasopharyngeal carcinoma**.
   - **Oropharyngeal carcinoma** (e.g., tonsil, tongue base).
   - **Hypopharyngeal carcinoma**.
   - **Laryngeal carcinoma**.
2. **Upper Aerodigestive Tract Cancers**:
   - Esophagus.
   - Trachea.
3. **Thyroid Cancer**:
   - Papillary or medullary carcinoma.
4. **Salivary Gland Cancer**:
   - Minor salivary glands.
5. **Distant Primary Tumors**:
   - Lung carcinoma.
   - Breast carcinoma.
   - Gastrointestinal tract cancers (e.g., stomach).
6. **Skin Cancer**:
   - Melanoma or squamous cell carcinoma.
7. **Lymphoma**:
   - Hodgkin's or Non-Hodgkin's lymphoma presenting with neck node involvement.

Management

1. Detailed History and Examination

- **History**:
  - Smoking, alcohol use, family history of cancer.
  - Symptoms: dysphagia, hoarseness, weight loss.
- **Examination**:
  - **Neck nodes**: Size, location, number, and consistency.
  - Thorough examination of **oral cavity**, **nasopharynx**, and **oropharynx**.

2. Investigations

Basic Investigations:

- **FNAC (Fine Needle Aspiration Cytology)**: Confirms metastatic nature and cell type.
- **Biopsy**: Excisional or core biopsy of the node if FNAC is inconclusive.

Advanced Imaging:

- **CT Scan or MRI**: Head, neck, and chest for primary site and node evaluation.
- **PET-CT**: Helps identify occult primary site.

Endoscopy:

- **Panendoscopy**: Direct visualization of oropharynx, nasopharynx, hypopharynx, and larynx.

Specialized Tests:

- **Immunohistochemistry (IHC)**: Identifies tumor origin.
- **EBV or HPV Testing**: For nasopharyngeal or oropharyngeal carcinoma.

3. Treatment

1. **Radiotherapy**:
   - Empirical treatment covering **potential primary sites** (e.g., nasopharynx, oropharynx).
2. **Surgery**:
   - **Neck dissection**: Removal of involved lymph nodes.
3. **Chemotherapy**:
   - Used in combination with radiotherapy for advanced cases.
4. **Follow-up and Monitoring**:
   - Regular imaging and examination to detect primary tumor or recurrence.

Key Points

- **Nasopharynx and oropharynx** are common sites for occult primaries.
- **Multimodal treatment** (surgery, radiotherapy, chemotherapy) improves survival.
- Early **PET-CT** can help detect hidden primary tumors effectively.

Q7. Differential Diagnosis of Abscess and Cellulitis

Abscess

- **Definition**:
  A **localized collection of pus** due to infection.
- **Features**:
  - **Localized swelling**.
  - **Fluctuant** (soft, compressible on palpation).
  - **Painful**.
  - May have **discharge** of pus.
  - Surrounding **erythema** (redness).
  - May show **pointing** (thinning of overlying skin).
- **Investigations**:
  - **Ultrasound**: Shows a fluid-filled cavity.
  - **Pus culture and sensitivity**: Identifies causative organism.
- **Treatment**:
  - **Incision and drainage**.
  - **Antibiotics** if cellulitis is also present.

Cellulitis

- **Definition**:
  A **diffuse infection** of skin and subcutaneous tissue without pus collection.
- **Features**:
  - **Diffuse redness** and **swelling**.
  - **Firm on palpation**, no fluctuation.
  - **Tenderness** over the affected area.
  - Skin is **warm to touch**.
  - **Fever** and systemic signs may be present.
  - Edges are **poorly demarcated** (blends into normal skin).
- **Investigations**:
  - **Complete blood count (CBC)**: Elevated white blood cell count.
  - **Blood cultures**: To rule out bacteremia.
- **Treatment**:

- **Antibiotics** (oral or IV, e.g., amoxicillin-clavulanic acid).
- Elevation of the limb to reduce swelling.

Key Differences

Feature	**Abscess**	Cellulitis
Nature	Localized pus collection	Diffuse infection
Fluctuation	Present	Absent
Swelling edges	Well-defined	Poorly defined
Systemic Signs	May be mild	Prominent (fever, chills)
Skin condition	Pointing or discharging	Red, warm, diffusely swollen
Management	Drainage + Antibiotics	Antibiotics alone

Differential Diagnosis of Non-Healing Ulcer

1. Malignant Causes

- **Squamous cell carcinoma**:
  - **Everted edges**, indurated base.
  - Common in smokers, betel nut chewers.
  - Non-healing for weeks/months.
  - May bleed on touch.
- **Basal cell carcinoma (BCC)**:
  - **Rolled edges**, pearly appearance.
  - Rarely metastasizes.
- **Melanoma**:
  - Ulceration with **pigmented lesion**.
  - Irregular edges, may bleed.

2. Infectious Causes

- **Tubercular ulcer**:
  - **Undermined edges**, shallow base.
  - Associated with **pulmonary tuberculosis**.
  - Painful with enlarged lymph nodes.
- **Syphilitic ulcer**:
  - **Snail track ulcers** in secondary syphilis.
  - Painless with firm edges in **primary syphilis (chancre)**.
- **Leprosy**:
  - Painless ulcers on anesthetic skin.
  - May involve nerves and cause deformity.
- **Fungal infections**:

- Seen in immunocompromised patients (e.g., **mucormycosis**).
- Irregular, necrotic ulcers with discharge.

3. Vascular Causes

- **Diabetic foot ulcer**:
  - Painless ulcer due to **neuropathy**.
  - **Deep with punched-out edges**, often infected.
- **Venous ulcer**:
  - **Superficial**, occurs over the medial malleolus.
  - Irregular shape with granulation tissue.
- **Arterial ulcer**:
  - **Painful** with a **punched-out appearance**.
  - Occurs on toes, heel, or pressure points.

4. Traumatic Causes

- **Traumatic ulcer**:
  - History of injury (e.g., sharp tooth, ill-fitting dentures).
  - **Painful**, heals after removing the cause.

5. Autoimmune Causes

- **Pemphigus vulgaris**:
  - Painful ulcers with surrounding blisters.
  - Fragile skin with **Nikolsky sign positive**.
- **Lichen planus**:
  - Ulcers with **white lacy patterns** (Wickham striae).
- **Systemic lupus erythematosus (SLE)**:
  - Painless oral ulcers associated with systemic features.

6. Other Causes

- **Radiation-induced ulcer**:
  - History of radiotherapy.
  - Ulcer with **fibrosis and necrosis**.
- **Drug-induced ulcer**:
  - NSAIDs or chemotherapy drugs.

Key Investigations

- **Biopsy**: To rule out malignancy.
- **Blood tests**: CBC, ESR, and autoimmune markers.

- **Microbiological tests**: Culture for infections.
- **Imaging**: X-ray or Doppler for vascular causes.

Management depends on the underlying cause and involves a multidisciplinary approach.

Q8. Differential Diagnosis of Non-Healing Ulcer.

• **Infected ulcer**:

- Caused by **bacteria**, **fungi**, or **viruses**
- Results in **delayed healing** due to persistent infection
- Appears **red, swollen**, with **purulent discharge**

• **Malignant ulcer**:

- Indicates **cancer** like **squamous cell carcinoma** or **basal cell carcinoma**
- **Chronic, non-healing**
- **Indurated**, **hard**, and **irregular** borders with **necrotic tissue**

• **Tubercular ulcer**:

- Caused by **Mycobacterium tuberculosis**
- **Painless** with **undermined edges**
- **Granulomatous base**

• **Venous ulcer**:

- Caused by **venous insufficiency**
- Located on the **lower leg**
- **Sloping edge** with **reddish base** and surrounding **discoloration**

• **Arterial ulcer**:

- Caused by **arterial blockages**
- **Painful**, **pale** or **necrotic base**
- Located over **pressure points**

• **Diabetic ulcer**:

- Common in **diabetic** patients
- Found on the **feet**
- Caused by **neuropathy** and **vascular compromise**

• **Pressure ulcer (Bed sore)**:

- Caused by prolonged **pressure** on **bony prominences**
- Seen in **bedridden patients**
- **Necrotic tissue**, common on **sacrum, heels, and hips**

• **Pyoderma gangrenosum**:

- **Painful ulcers** with **purulent discharge**
- **Violaceous borders**
- Associated with **systemic diseases** like **inflammatory bowel disease**

• **Chronic venous leg ulcer**:

- Caused by **venous hypertension**
- **Shallow** with **irregular borders**
- **Yellowish, sloughy base**

• **Fungal ulcer**:

- Caused by **fungal infections** like **Candidiasis**
- **Moist**, **red**, **inflamed**, with **white coating**

• **Herpetic ulcer**:

- Caused by **herpes simplex virus**
- Appears as **grouped vesicles**
- **Painful**, **shallow ulcers** with **crusted edges**

• **Venous leg ulcer**:

- Caused by **chronic venous insufficiency**
- Typically located on the **lower legs** near the **ankles**
- **Irregular edges**, **discoloration**, and **swelling**

• **Traumatic ulcer**:

- Caused by **physical injury** like cuts, burns, or abrasions
- Leads to **open wound**, which may get infected

• **Autoimmune ulcer**:

- Seen in **lupus** or **bullous pemphigoid**
- **Blistering**, **erosions**, and **painful lesions**
- May recur and take time to heal

• **Ischemic ulcer**:

- Caused by **poor circulation** due to **diabetes** or **atherosclerosis**
- **Painful**, **pale** or **bluish base**
- **Necrotic tissue**

• **Traumatic ulcer**:

- Caused by **cuts**, **abrasions**, or **burns**
- Exposes underlying skin, causing a **painful wound**
- Risk of **infection** if untreated